

Does Low Medical Spending Mean Low Medical Financial Risk?

Sound Money Stress Tests, Essay 6.

Arne Kotte

AUTHOR

Arne Kotte

DOCUMENT TYPE

Contrarian essay

DATE

May 2026

Does Low Medical Spending Mean Low Medical Financial Risk?

The objection

A serious objection to medical-financial-fragility research is that spending is observable. If a person has low out-of-pocket medical spending, why describe medical care as a financial risk? Maybe low spending simply means insurance is working, medical need is low, or the household is not materially exposed.

That objection matters because it prevents a common overreach. High medical spending can signal burden, but low medical spending is not automatically hidden hardship. Many people spend little because they are healthy, well insured, or protected by public programs. A careful analysis should not treat every low-spending case as foregone care.

The question is narrower: can realized medical spending, by itself, classify medical financial risk?

What this essay means by medical financial risk

This essay uses **medical financial risk** to mean exposure to medical needs that can strain a household through out-of-pocket bills, premiums, cost-related delayed care, skipped prescriptions, unaffordable visits, or other financial barriers to obtaining care.

That definition is intentionally broader than realized out-of-pocket spending, but narrower than overall health status or overall healthcare quality. It does not mean every access problem is financial. Provider shortages, transportation, appointment delays, network limits, preferences, time constraints, and perceived severity can also affect care use.

The measurement question is therefore about classification validity: how well does observed spending classify people who may face medical financial vulnerability?

Why spending alone is ambiguous

Medical spending is a realized outcome. It reflects at least three different things:

- **Need:** whether someone required care.
- **Protection:** whether insurance, public coverage, or provider arrangements reduced out-of-pocket exposure.

- **Access:** whether someone could obtain care when care was needed.

Those three channels can produce the same observed spending number.

Low out-of-pocket spending can be good news if it reflects low medical need or strong insurance protection. It can also be concerning if it reflects delayed care, skipped prescriptions, unaffordable visits, or inability to access providers. Spending alone cannot distinguish those cases.

High out-of-pocket spending is also not simple. It may reflect serious burden, but it may also reflect access to needed care, use of services, or a temporary episode that a household can absorb. A dollar amount has to be read with income, insurance status, health need, and access barriers.

A simple classification map

The basic measurement problem can be stated as a four-cell map:

Spending/access pattern	Plausible interpretation
Low spending, no access barrier, low need	likely benign low spending
Low spending, cost-related access barrier, nontrivial need	possible unmet need or hidden financial vulnerability
High spending, high income or strong buffer, no access barrier	spending without clear fragility
High spending, low income or thin buffer, access barrier	conventional burden risk

The point is not that the table proves who is fragile. It is that spending-only measures collapse these cases. A person with low spending and no access problem is not in the same situation as a person with low spending because care was delayed or unaffordable.

That is why the right target is not “low spending equals risk.” The better target is: spending alone is an incomplete classifier.

What the evidence can test

Public and public-use medical expenditure data can test whether spending, insurance, health status, and access barriers tell the same story or different stories.

The relevant question is not only “How much did people spend?” It is also:

- Who had medical need?
- Who had insurance protection?
- Who delayed or skipped care because of cost?
- Whose spending burden was high relative to income?
- Whose low spending coincided with access barriers?

That is a descriptive measurement question, not a causal one. It cannot prove that insurance status caused delayed care, or that a specific out-of-pocket burden caused hardship. It can show whether spending-only measures miss people whose medical financial vulnerability is visible in access barriers rather than in dollars spent.

What the argument should concede

The argument should concede three things.

First, low medical spending is often benign. People who are healthy, well insured, or protected by low cost-sharing may have low out-of-pocket spending for good reasons.

Second, high spending is not automatically fragility. A household with high income, strong savings, good insurance, and temporary medical needs may absorb a large bill without broader hardship.

Third, medical expenditure data are technically difficult. Family construction, premium treatment, survey design, timing, income denominators, and questionnaire continuity all affect interpretation. A working paper should not pretend those issues are fully settled if they are not.

Those concessions are central. The goal is not to claim that low spending proves hidden risk. The goal is to show why low spending cannot, by itself, prove low risk.

What the MEPS evidence suggests

The MEPS medical-financial-fragility work studies out-of-pocket medical burden, insurance status, health need, and delayed or unaffordable care together.

Its strongest descriptive result is the spending/access ambiguity. In MEPS 2022, uninsured people have lower observed out-of-pocket spending than privately insured people but much higher delayed or unaffordable-care rates. That pattern is exactly why spending alone is not enough.

The draft also finds that lower-income groups can face higher out-of-pocket burden shares even when their dollar spending is lower, and that poor-health groups face both higher medical spending and more access barriers. Robustness checks across 2019 through 2023 suggest that the basic spending/access ambiguity is not isolated to a single year.

These statements summarize the companion MEPS working paper rather than reproducing its full tables here. They should be read as descriptive measurement evidence. They do not identify causal effects of insurance status, plan design, or medical prices. They do show that realized spending and medical financial risk are not the same object.

Insurance complicates the interpretation

Insurance can lower observed out-of-pocket spending by protecting households. That is the point of insurance.

But insurance can also make interpretation harder. Premiums, deductibles, copayments, networks, covered services, denied claims, and provider availability all shape whether a household is financially protected in practice. A person may have low out-of-pocket spending because coverage is strong. Another may have low spending because care was not obtained. A third may have moderate spending but high premium burden.

That is why medical financial risk should not be measured with one number. A better dashboard separates:

- out-of-pocket spending;
- premiums where they can be measured responsibly;
- income or liquid resources where available;
- insurance status and coverage type;
- health need;
- cost-related delayed or unaffordable care;
- broader access barriers.

The MEPS work is useful because it points toward that dashboard, while still labeling unfinished pieces as unfinished.

The burden-share problem

Dollar spending alone also misses burden. A \$500 medical bill does not mean the same thing for a high-income household with savings as for a low-income household with little cash buffer.

That is why burden shares matter. But burden shares have their own measurement problems. Very low income denominators can produce unstable ratios. Family-level construction matters. Premium annualization and linkage can change the interpretation. Survey weights and design choices matter.

The right conclusion is not to abandon burden measures. It is to treat them carefully. Dollar spending, burden shares, insurance status, and access barriers should be read together.

This essay should also avoid pretending MEPS directly observes full household resilience. MEPS can observe spending, insurance, health status, income, family structure, and reported access barriers. It does not fully observe liquid savings, informal support, or every household balance-sheet backstop.

What this does not prove

This evidence does not prove that every uninsured person with low spending is foregoing care. Some uninsured people may have low need. Some may use low-cost clinics, community resources, uncompensated care, or temporary arrangements not fully captured by a simple spending interpretation.

It also does not prove that insured people are financially safe. Coverage can reduce risk, but high deductibles, premiums, narrow networks, and denied or delayed care can still create financial exposure.

Nor does it prove a single cause of medical financial fragility. Health status, insurance design, income, family structure, geography, provider access, employment, public programs, and household choices all matter.

Finally, delayed care is not always financial fragility. Some delayed care reflects nonfinancial access problems, time constraints, provider availability, preferences, or perceived low severity. The financial-risk claim is strongest when spending measures are read alongside cost-related access barriers and health need.

The narrow conclusion

The strongest answer to the objection is to accept that low spending can be good news and then ask what kind of low spending it is.

If low spending reflects low need or strong protection, it suggests lower medical financial risk. If low spending reflects delayed care, unaffordable treatment, skipped prescriptions, or lack of access, it may fail to capture risk rather than disprove it.

The defensible claim is modest: low observed medical spending does not, by itself, prove low medical financial risk. Medical financial vulnerability is better treated as a classification problem that requires spending, income, insurance, health need, and access barriers together.

Evidence note

This essay relies on the MEPS medical-financial-fragility working paper, the fragility-premium measurement paper, and the household-resilience capstone. The figures and patterns cited here are descriptive and should not be read as causal estimates.

A formal empirical paper would need fully documented MEPS variable construction, survey-design treatment, family-unit validation, premium-linkage validation, timing checks, access-barrier outcome definitions, burden-share denominator rules, and tables showing magnitudes by

insurance, income, health status, and access-barrier group. The current MEPS work is useful because it states those boundaries rather than hiding them.

Related research-library pieces

- *Medical Financial Fragility: A MEPS Working Paper on Cost Burden, Insurance, and Access Barriers*
- *Society Under Monetary Stress IX: The Fragility Premium*
- *Mapping Household Financial Resilience*